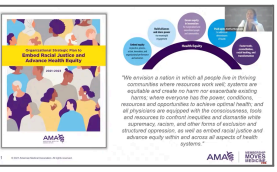
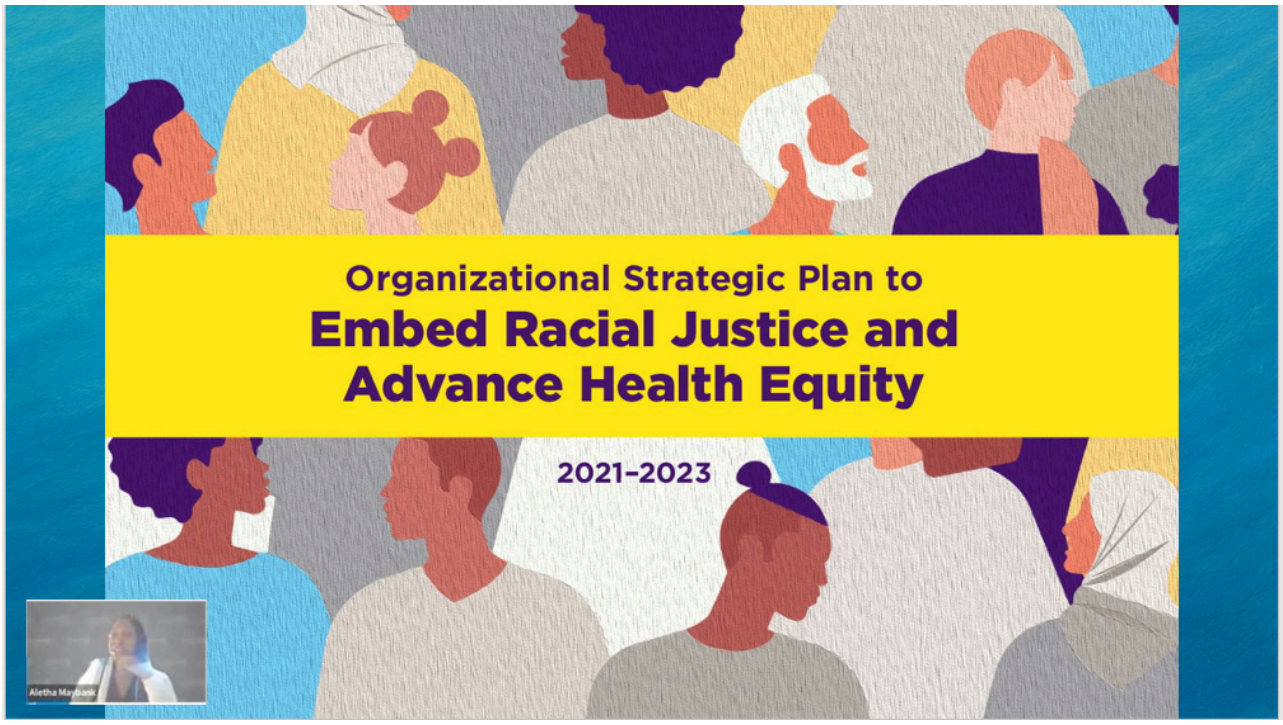
Webinar Redesign

Revising the webinar slides using ideas from UBD and cognitive science strengthened my ability to make learning materials clearer, more focused, and more respectful of learners' cognitive load.

SLIDE 1 REVISION

BEFORE	AFTER
	Slide 1A  Slide 1B



NOTES:

According to Mayer's ***Coherence Principle****, people learn better when extraneous material is excluded so that the main message stands out more clearly.* I noticed that slide 1 contained words and images that did not directly support learning, so I applied this principle as follows. This first slide introduces AMA's five strategic approaches to advancing health equity. Dr. Maybank's commitment is clear, and her message deserves full attention. Yet showing the vision statement, plan cover, and five approaches diagram all at once adds unnecessary visual load, distracting viewers from the key message. When slides include extra text or visuals that don't directly support the speaker's message, learners must work harder to filter what matter–fragmenting attention and breaking the overall flow of ideas. I noticed Dr. Maybank doesn't mention the vision statement, so it ends up competing with her speech rather than supporting it. Therefore, I excluded the vision statement to keep the focus on Dr. Maybank's message. To reduce visual clutter, I removed extra elements like the copyright statement and logos, helping learners concentrate on the core message. With less extraneous text and graphics, participants can really tune in to Dr. Maybank's main points and understand the significance of the strategic plan.

The ***Signaling Principle*** *means that people learn better when visuals and narration give clues about what to focus on and how ideas connect.* The original first slide lacked a clear focus and visual cues to guide attention, so I applied this principle to make the organization easier to follow. First, I split the content into two slides to focus the learner's attention. On Slide 1A, I used the report cover as the main visual, which already displays the title prominently and naturally signals the focus of the slide. I added a clear title to Slide 1B to signal the topic shift and help learners mentally organize the content. As Dr. Maybank says, "so these are the five approaches that we have," the presenter should transition to Slide 1B. Switching slides at that moment helps learners understand the structure of her message and not get lost. The presenter can highlight each shape as Dr. Maybank mentions it, so

everyone knows which point she’s talking about. This highlighting guides attention, supports understanding, and keeps the viewer from feeling overloaded.

SLIDE 2 REVISION

BEFORE	AFTER
	

NOTES:

People learn more effectively when visuals and words work together, so I applied Mayer's ***Multimedia Principle*** *to this slide, which originally had no images.* The original second slide covers a sensitive and important topic–AMA's historical role in racial injustice within medicine. Dr. Maybank's openness about this history shows integrity and courage. But the slide relied on text-heavy bullet points, making it hard for learners to follow the timeline or remember key events. To apply Mayer's Multimedia Principle, I replaced the text list with a horizontal timeline of five colored shapes, each paired with a short phrase and a simple white line drawing that illustrates the event. Pairing images with spoken language helps the brain make sense of complex information more effectively than using either alone, making it easier for learners to follow the timeline as they listen to Dr. Maybank. Because visuals are often easier to recall than text alone, the icons help make these historical events more memorable. Each shape is highlighted as Dr. Maybank talks about that event. This ties her words to each visual, helping learners stay on track. This coordination helps learners link what they hear to what they see, activating both the brain's visual and auditory pathways so the timeline is easier to follow. Adding visuals that support understanding while the narration provides context reduces cognitive load and gives medical professionals mental space to absorb this vital message about health equity.

*Mayer's **Modality Principl**e highlights that people understand better when they hear narration alongside visuals, instead of trying to read a lot of text while someone's talking.* The second slide relied heavily on written text while the speaker was actively narrating. This overlap can split learners' attention, making it harder to absorb either the spoken or written content. So I applied this principle by adding graphics that support what the speaker is saying, and reducing the text. To support better processing, I replaced the text-heavy bullet points with a visual timeline that pairs short, clear phrases with simple icons. This lets the narration give background for each event while the slide graphics help the audience hold onto the key points. To implement the modality principle, each circle briefly lights up as the speaker mentions it, helping viewers follow along and focus on the right point in the timeline. This gentle cue connects the narration with the visual at the same moment, reducing mental effort and allowing both sound and image to work together smoothly. The timeline's structure encourages viewers to listen and look–rather than read and listen–so the brain's verbal and visual pathways work together instead of competing. In a topic this emotionally charged, it felt especially important to reduce cognitive load and give time to take in the message. This design not only clarifies the content but also shows respect for the emotional weight of the history being presented.

Reflection

In 2–6 sentences, thoughtfully answer each of the questions below:

1. Where did you get stuck when working on this project?

A. It seemed like everything had to be processed at once: the original slide text, the speaker's narration, the old slide, the new slide, Mayer's 12 principles, the project description, criteria from the readings, the interview situation, principles of emotional intelligence, and the available graphics.

2. What did you do to get unstuck?

A. First, I just designed a slide using one of Mayer's 12 principles. Then I evaluated the revised version by systematically examining each criterion and piece of contextual information–one at a time–including the original slide text, the speaker's narration, the old slide, the new slide, Mayer's 12 principles, the project description, criteria from the readings, the interview situation, principles of emotional intelligence, and the available graphics–comparing each with my revised slide and revising the slide again as needed.

Project 3: Cognitive Wrapper

Complete the following reflection within a few days of receiving the evaluation and feedback for your project. Your responses have no impact on your grade and are solely to help you improve. We encourage you to be brutally honest with yourself in order to get the most out of the reflection. We will not assess this. We will briefly glance at it to confirm that you completed it with thoughtfulness; shallow cognitive wrappers will be returned to complete again.

Review your grade and feedback, answer the four questions below:

1. What is the strongest aspect of your project—what did you do really well? State in your own words.

Clear communication both visually and in words.

2. What are the most significant aspects to revise? State in your own words and be really specific, e.g., “Instead of X for the big idea, I would choose Y” instead of “I would find a better big idea.” (Note: This will help you a lot in Block 6!).

If I were to revise this, I would replace the general smiley icon with a smiley depicting a doctor whose mouth is zipped shut. This adjustment would convey a more professional tone.

3. Which of the following contributed to difficulties you had on the project? Check all that apply.

Didn’t complete the Self-Paced module in a timely manner	
Didn’t complete the required readings in a timely manner	
Didn’t not actively participate in the small group meeting	
Didn’t attend small group meeting	
Trouble understanding the underlying concepts	
Trouble applying the concepts	
Did not read and follow the project requirements closely enough	
Time management (didn’t spend enough time or procrastinate or personal issues)	
Careless mistakes	
Minimal effort	
Overthinking/analysis paralysis	
Did not seek answers to questions, either trying to figure it out yourself, asking your small group, or asking the full class via Canvas discussion board or the Live Session	

4. What will you do differently to prepare and complete the next project? Be specific. For example, will you spend more time, complete the Self-Paced module earlier, complete the readings earlier, post specific questions to the Discussion Board, participate more in the small group meeting, reach out to your small group and ask for a second meeting, etc.?

If I choose to use a smiley icon, I will select one that reflects the profession I am serving.